

Pediatric
P-02 Neonatal Resuscitation BLS

Revised 3/1/2000

PRIORITIES:

- Assessment;
- Vital signs (normal ranges – heart rate: 120-190; respiratory rate >40);
- APGAR Rating;
- Assure an advanced life support response;
- All neonates need to be gently suctioned (mouth first, then nose), dried and kept warm;
- Neonates in full arrest and neonates not initially responding to treatment should be transported as soon as possible with CPR in progress;
- Begin CPR if heart rate is less than 80/min., apneic, limp, cyanotic or APGAR 6 or less.

APGAR SCORE

Check APGAR at 1 minute, 5 minutes & every 5 minutes thereafter.

APGAR CHART	0	1	2
Appearance	Blue-Pale Limbs blue	Body Pink	Pink all over
Pulse	0	>100	100>
Grimace	No Response	Grimace	Cough, Cry, Sneeze
Activity	Flaccid	Some Flexion	Active Movement
Resp. Effort	Absent	Slow, Irregular	Strongly Crying

APGAR 7 – 10

1. Suction with bulb syringe
2. Keep dry warm (skin to skin with mother and blanket)
3. Initiate transport when available.

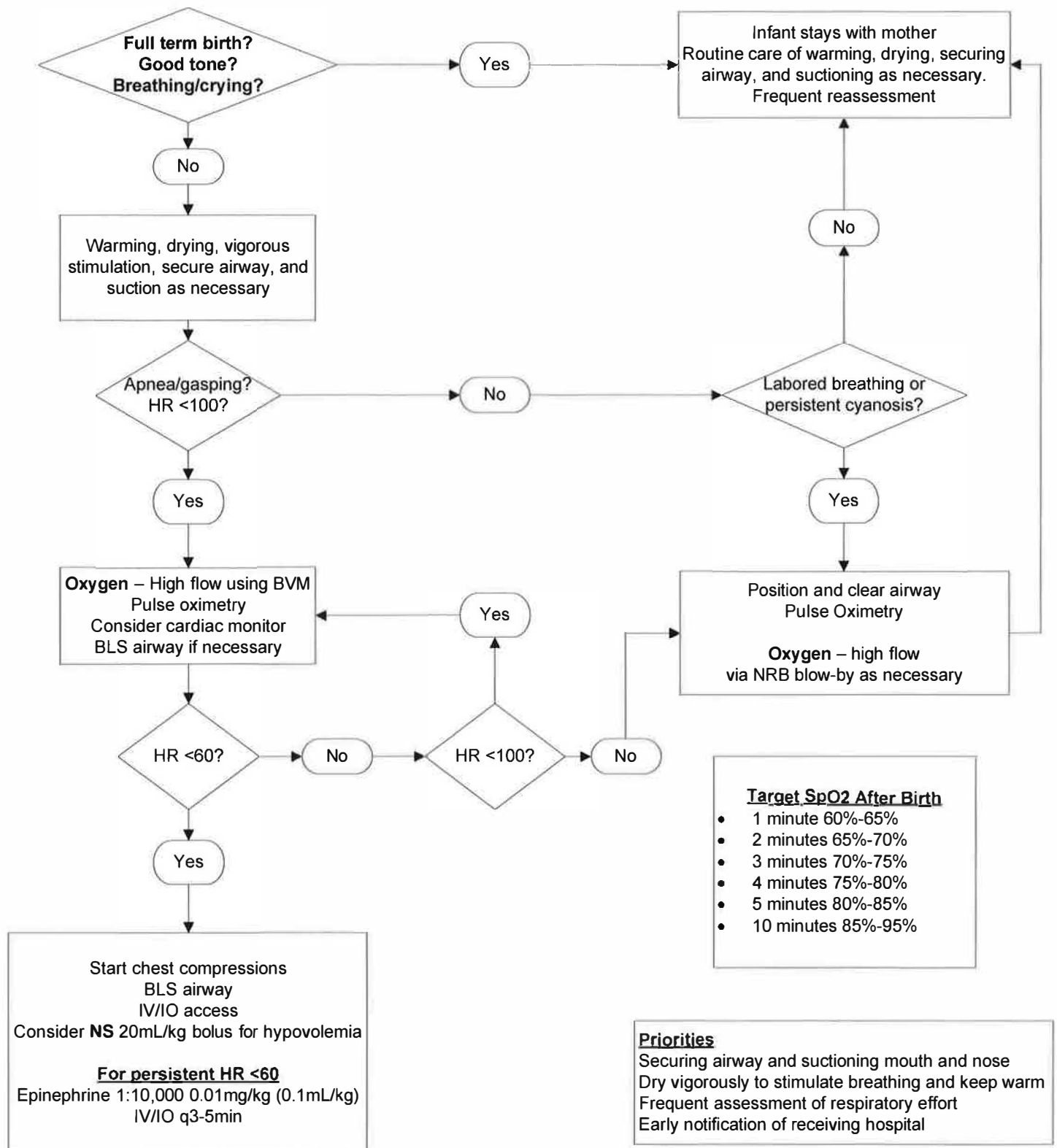
APGAR 4 – 6

1. Suction with bulb syringe;
2. Ventilate 30 – 40 breaths/min with 100% OXYGEN, bag and mask;
3. Keep dry and warm;
4. Initiate immediate transport when available.

APGAR 0 – 3

1. Suction with bulb syringe;
2. Support ventilation 30 – 40 breaths/min with 100% OXYGEN, bag and mask;
3. If heart rate < 80/minute, begin cardiac compression;
4. Keep dry and warm;
5. Initiate immediate transport when available.

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DISRUPTED COMMUNICATIONS

In the event of a “disrupted communications” situation, Solano County Paramedics may utilize all portions of this treatment protocol without Base Hospital Contact as needed to stabilize an immediate patient.